IT DUE DILIGENCE REPORT

CENTER FOR ADVANCED SURGICAL SPECIALISTS

Tampa, Florida

Prepared for Compass Surgical Partners
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Executive Summary

RECOMMENDATION: PROCEED WITH CONDITIONS

The Center for Advanced Surgical Specialists (CASS) presents a generally favorable IT posture for acquisition. The facility operates on Cisco Meraki infrastructure, which aligns with Compass standards and simplifies integration. The building was purpose-built in 2017-2019 with fiber backbone and modern structured cabling. However, several gaps require attention: the current IT provider operates as a part-time engagement with limited documentation, security controls could not be verified during the visit, and the Meraki MX84 firewall creates a throughput bottleneck. These are manageable issues with a structured Day 1 and stabilization plan.

Critical Findings	High Findings	Est. CapEx	Est. Annual OpEx
0	**2**	**$56,000 - $72,500**	**$72,000 - $96,000**

Conditions for Proceeding:

1. Follow-up meeting with Lance (IT provider) must be completed to verify security controls, backup configuration, and documentation status before close.
2. Seller to provide all IT credentials, vendor contacts, and system documentation at or before close.
3. Seller to confirm status of MFA, EDR, and backup solutions currently deployed (if any).

Critical Pre-Close Requirements:

- Obtain all admin credentials for Meraki dashboard, server, firewall, switches, and wireless controllers.
- Obtain vendor contact information for ISP, Amkai, iPro, IDEX, VoIP provider.
- Complete IT discovery call with Lance to fill documentation gaps.
- Execute BAA with Dedicated IT for Day 1 support.

Facility Overview

Facility Name	Center for Advanced Surgical Specialists (CASS)
Location	Tampa, Florida
Approximate Staff	~25-30 (estimated from observation)
Endpoints	~25-35 (estimated - workstations, OR mini PCs, printers)
Operating Rooms	4
Specialties	Spine, Orthopedics, Pain Management, Pediatrics, ENT, Oculoplastics
IT Contact	Lance (surname unknown) - External IT; unavailable during visit. Manages multiple companies; This center is not his exclusive focus.

Findings Summary

ID	Finding	Risk	Phase	Est. Cost
F001	IT Provider Dependency / No Documentation	**HIGH**	DAY1	$8,500
F002	Security Controls Unverified (MFA/EDR/Backup)	**HIGH**	WEEK1	$11,000
F003	Meraki MX84 Firewall Throughput Limitation	**MEDIUM**	STABILIZE	$6,700
F004	No Dedicated PACS System	**MEDIUM**	STABILIZE	$3,000 - $8,000
F005	UPS Battery Failure in Secondary IDF	**MEDIUM**	WEEK1	$500
F006	Consumer-Grade Security Cameras (Arlo)	**LOW**	OPTIMIZE	$0 (evaluate)
F007	Missing Wireless Access Point	**LOW**	WEEK1	$950
F008	Paper-Based Clinical Documentation	**LOW**	OPTIMIZE	N/A

Detailed Findings

HIGH Risk Findings (2)

F001: IT Provider Dependency / No Documentation	
Description	The facility's IT is managed by Lance, an external provider who operates multiple companies and provides IT services on a part-time basis. Lance was unavailable during the site visit and had not responded to messages as of the time of departure. No IT documentation, network diagrams, or system credentials were provided or available on-site. Staff indicated Lance 'is always playing with stuff' and that they 'don't mess with' his equipment.
Business Impact	Operational: Without documentation or credential access, Compass would be unable to respond to any IT incident post-close, potentially causing extended downtime during surgical operations. Financial: Emergency vendor engagement to recover access could cost $5,000-$15,000. Compliance: HIPAA requires documented IT processes and business associate agreements; the current arrangement likely does not meet these requirements.
Remediation	1. Complete follow-up meeting with Lance pre-close to document all systems, credentials, and vendor contacts. 2. Execute Day 1 Takeover including credential rotation, admin access establishment, and comprehensive documentation. 3. Establish BAA with Dedicated IT and terminate or formalize relationship with Lance.
Timeline / Cost	DAY1 - $8,500 (Day 1 Takeover: 40 hrs @ $175/hr + PM)
Evidence	OBSERVED (Lance unavailable) / REPRESENTED (staff comments about Lance's role)

F002: Security Controls Unverified (MFA / EDR / Backup)	
Description	Due to Lance's unavailability, the status of critical security controls could not be verified during the site visit. It is unknown whether Multi-Factor Authentication (MFA) is deployed on any accounts, whether an Endpoint Detection and Response (EDR) platform is in use, or what backup solution protects the on-premise server and critical data. The HP DL380 Gen9 server was observed in the main IT closet, but its backup status is unknown.
Business Impact	Operational: Without MFA, a single compromised credential could provide access to the EMR, scheduling system, and patient data. Without EDR, ransomware could propagate undetected. Without verified backups, data loss from any incident could be unrecoverable. Financial: Healthcare breach costs average $400+ per record; even a small breach at this facility could result in $500K+ in costs. Compliance: MFA and backup are fundamental HIPAA Security Rule requirements.
Remediation	1. Verify current state with Lance during follow-up call. 2. Deploy MFA across all user accounts (Microsoft Entra ID or Duo). 3. Deploy EDR to all endpoints (SentinelOne or CrowdStrike). 4. Verify or implement Datto BCDR for server and critical data backup.

Timeline / Cost	WEEK1 - $11,000 (Security Baseline: MFA deployment $4,000 + EDR deployment $7,000 for ~30 endpoints)
Evidence	UNVERIFIED - Could not confirm presence or absence of any security controls

MEDIUM Risk Findings (3)

F003: Meraki MX84 Firewall Throughput Limitation	
Description	The facility has a Cisco Meraki MX84 firewall observed in the main IT closet. The facility is paying for approximately 2 Gbps internet service, but the MX84 has a maximum stateful firewall throughput of approximately 500 Mbps. This means the facility is paying for bandwidth it cannot utilize. The MX84 is also approaching end-of-sale status.
Business Impact	Operational: The facility is not experiencing the internet performance it is paying for, though current operations appear unaffected. Financial: The facility is overpaying for internet bandwidth that cannot be utilized. As the MX84 approaches end-of-life, it will lose firmware updates and security patches. Integration: Replacing with a current-generation Meraki MX maintains Compass standards alignment.
Remediation	Replace MX84 with Meraki MX68 (supports up to 450 Mbps with advanced security features and current licensing) or MX85 if higher throughput is needed to leverage the 2 Gbps circuit. Review ISP contract to right-size bandwidth to match firewall capabilities or upgrade firewall to match bandwidth.
Timeline / Cost	STABILIZATION - $6,700 (MX68 hardware $2,400 + 3yr license $1,500 + implementation 16hrs @ $175/hr = $2,800)
Evidence	OBSERVED - MX84 physically identified in rack; throughput limitation noted by assessor

F004: No Dedicated PACS System	
Description	The facility does not have a dedicated PACS (Picture Archiving and Communication System). Imaging from C-arm fluoroscopy is stored locally on the machines themselves, with copies stored in Amkai and on flash drives. Staff indicated images are retained approximately one year on the C-arm before being overwritten. Two analog machines and one digital machine are in use.
Business Impact	Operational: Loss of a C-arm or its storage could result in permanent loss of surgical images. Images needed for follow-up surgery or medical records requests may be unavailable. Compliance: Medical image retention requirements vary by state but typically exceed one year. Financial: Implementing a cloud PACS would cost $200-$400/study but eliminates storage risk and provides universal access.
Remediation	Evaluate cloud-based PACS solution (e.g., Ambra Health, PostDICOM) for centralized image archiving. Configure C-arms to auto-send images to PACS

	via DICOM. This provides long-term retention, physician mobile access, and compliance with records retention requirements.
Timeline / Cost	STABILIZATION - $3,000-$8,000 (DICOM relay setup + annual subscription based on study volume)
Evidence	REPRESENTED - Staff confirmed images stored locally on C-arms and in Amkai

F005: UPS Battery Failure in Secondary IDF	
Description	A secondary network rack located in the medical equipment storage room was observed with UPS battery issues. Audible beeping was noted, and a dead battery unit was identified. Rather than replacing the failed unit, a new UPS was stacked on top of the old one. The main IT closet UPS (Eaton) appeared functional at ~14% load.
Business Impact	Operational: Equipment in this IDF has no battery protection during power events, risking data corruption or hardware damage. The dead UPS is a fire hazard (swollen/failed batteries). Financial: Minor cost to resolve but risk of cascading equipment failure if ignored.
Remediation	Remove dead UPS unit and properly dispose of batteries. Verify replacement UPS is adequately sized for the load. Ensure all network equipment is connected to protected outlets.
Timeline / Cost	WEEK1 - $500 (battery replacement and disposal, 2hrs labor)
Evidence	OBSERVED - Audible beeping heard; dead battery unit photographed; staff confirmed issue

LOW Risk Findings (3)

F006: Consumer-Grade Security Cameras (Arlo)	
Description	The facility uses Arlo consumer-grade wireless security cameras. These were observed in the medication room area and other locations. Arlo cameras are battery-powered, cloud-dependent, and not designed for healthcare facility use.
Business Impact	Arlo cameras have limited recording retention, depend on cloud connectivity and battery life, and lack integration with enterprise security systems. However, this is a low priority as the facility does have camera coverage in key areas.
Remediation	Evaluate during optimization phase whether to upgrade to enterprise camera system (Verkada, Meraki MV, or similar). Not urgent.
Timeline / Cost	OPTIMIZATION - $0 (evaluate only; upgrade if needed during facility improvements)
Evidence	OBSERVED - Arlo camera units photographed in medication area

F007: Missing Wireless Access Point	
Description	A Meraki AP mounting bracket was observed on a ceiling in the front area of the facility with no access point installed. It appears the AP was removed (possibly failed) and relocated from another area. Staff acknowledged the missing AP.
Business Impact	Minor coverage gap in the front area. Staff and physicians reported no WiFi complaints, suggesting adjacent APs may provide sufficient overlap. However, this should be verified with a wireless survey.
Remediation	Install replacement Meraki MR AP on existing bracket. Verify coverage with wireless survey during stabilization.
Timeline / Cost	WEEK1 - $950 (Meraki MR AP hardware $600 + installation $350)
Evidence	OBSERVED - Empty mounting bracket photographed; assessor noted gap

F008: Paper-Based Clinical Documentation	
Description	Clinical documentation is primarily paper-based and then scanned into the IDEX document management system. Physicians prefer paper. Anesthesia records, intraoperative records, and similar forms are manually handled.
Business Impact	This is an efficiency opportunity, not an immediate risk. Paper-based workflows increase labor costs for scanning and introduce potential for lost or misfiled records. However, physicians are comfortable with the current workflow and the scanning process is established.
Remediation	Evaluate tablet-based documentation during EMR/PM transition to HST. If Compass standardizes on HST, consider deploying tablets for direct EMR entry, eliminating the scan step.
Timeline / Cost	OPTIMIZATION - N/A (evaluate during EMR transition)
Evidence	REPRESENTED - Staff described paper-based workflow with scanning into IDEX

Infrastructure Summary

Network Infrastructure

Component	Details
Firewall	Cisco Meraki MX84 (throughput-limited, approaching EOL)
Switches	Cisco Catalyst 2960-X 48-port PoE 370W (x2) with fiber SFP modules
Wireless	Cisco Meraki MR series (~6-8 APs, 1 missing)
Internet	~2 Gbps (provider unknown - verify with Lance); bottlenecked by MX84
Server	HP ProLiant DL380 Gen9 - Role unverified (likely file server / local applications)
UPS	Eaton (main closet, 14% load, functional) + secondary UPS (bad battery, beeping)
Cabling	12-strand fiber to IDFs (1B, 2A); structured Cat5e/6 throughout; provider patch panel
Phone System	Cisco VoIP (in transition from legacy phones)
Cameras	Arlo consumer-grade wireless cameras
Generator	250kW backup generator

Applications

System	Solution	Hosting
EMR	iPro	Unknown - verify with Lance
Practice Management	Amkai (used for scheduling, billing, everything)	Unknown - likely cloud/hosted
PACS	None - images on C-arm machines	Local / On-device
Document Management	IDEX	Unknown
Email	Unknown	Unknown

Security Controls

Control	Status	Notes
Multi-Factor Auth	**UNKNOWN**	Could not verify - Lance unavailable
Endpoint Detection	**UNKNOWN**	Could not verify - no agent observed on visible screens

Email Security	**UNKNOWN**	Email platform itself unknown
Backup Solution	**UNKNOWN**	Server present but backup status unverified
Physical Security	**DEPLOYED**	Arlo cameras, access control on doors, locked server room

Cost Estimates

Capital Expenditures (Year 1)

Item	Amount	Notes
Day 1 Takeover	$8,500	40 hrs @ $175/hr + PM; credential rotation, vendor intros, documentation
Security Baseline	$11,000	MFA deployment ($4,000) + EDR deployment ($7,000) for ~30 endpoints
Firewall Replacement	$6,700	Meraki MX68 ($2,400) + 3yr license ($1,500) + install 16hrs ($2,800)
Wireless (1 AP)	$950	Replacement Meraki MR AP + installation
UPS Battery Repair	$500	Replace dead battery, dispose of old unit
Backup Enhancement	$12,000	Datto BCDR appliance + implementation (if no backup exists)
PACS Implementation	$5,000	Cloud PACS setup + DICOM relay configuration
Documentation	$5,000	30 hrs @ $175/hr - network diagrams, runbooks, credential vault
Subtotal	**$49,650**	
Contingency (15%)	$7,448	Higher contingency due to unverified systems
TOTAL CAPEX	**$57,098**	

Operating Expenditures (Monthly)

Item	Monthly	Notes
Managed Services	$4,500	~30 users @ $150/user (Standard tier)
M365 Licensing	$750	~30 users @ $25/user (Business Premium)
Backup Service	$750	Datto BCDR monthly service
Security Tools	$450	MFA ($4/user) + EDR ($7/endpoint) for 30 users/endpoints
TOTAL MONTHLY	**$6,450**	
TOTAL ANNUAL	**$77,400**	

Follow-Up Items

Item	Owner	Due Date	Status

Schedule follow-up call with Lance	Jay / Ben Posner	Pre-Close	**PENDING**
Obtain all IT credentials and vendor contacts	Jay / Lance	Pre-Close	**PENDING**
Verify MFA / EDR / Backup status	Ben Posner	Pre-Close	**PENDING**
Obtain ISP contract details	Jay	Pre-Close	**PENDING**
Confirm server role and OS version	Lance / Ben	Pre-Close	**PENDING**
Determine email platform and hosting	Lance / Ben	Pre-Close	**PENDING**

Deal Risks

- IT provider (Lance) has been unresponsive; if he remains uncooperative during transition, discovery and credential recovery could significantly increase Day 1 costs and extend timeline.
- Security posture is entirely unverified; if MFA, EDR, and backup are all absent, the security baseline cost could be $15,000-$20,000 higher than estimated.
- If the on-premise server is running unsupported OS (Windows Server 2012/2016), replacement costs of $15,000-$40,000 could be required.
- Email platform unknown; if on-premise Exchange is in use, migration to M365 would add $5,000-$8,000 to CapEx.
- Tribal knowledge risk is significant; Lance appears to be the only person who understands the IT environment. Loss of his cooperation could create extended operational risk.

Positive Observations

✓ Facility already runs Cisco Meraki infrastructure (switches, wireless, firewall), aligning with Compass standards and simplifying integration.
✓ Purpose-built facility (2019) with fiber backbone, structured cabling, and proper IDFs. Infrastructure is modern and well-organized in the main closet.
✓ Main IT closet is clean, well-ventilated, and properly secured with access control.
✓ Eaton UPS in main closet is functional and lightly loaded (14%), providing ample headroom.
✓ Staff familiar with Amkai and iPro; HST transition path is well-understood by Compass with prior successful migrations.
✓ 250kW backup generator provides full facility power redundancy.
✓ WiFi coverage reported as good throughout facility by staff and physicians.
✓ Physical access controls on doors tied to security system; medication room has camera and dual locks.

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